Confidential and Protected Communication

Fax

Send To:	MEADOWS HOME HEALT	**Sent From:**	Laura Cosio
To Fax #:	+1 702-405-4411	**From Phone #:**	+17026915614
Date:	04/17/2026 03:03:17 PM	**Pages:**	13

Message:

Anticipated DC: 04/17
PCP: Forte Family
Clinic: NA
Services:
Home Health: Meadows - 2RN - wound care - SOC 04/18
Home Infusion: NA
DME: NA
Other Services: Byram - wound care supplies
Thank you

Disclaimer:

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MED REC# 63844265

PT#	PATIENT NAME	CELL PHONE
1101376356	AJOLO, ANGEL SABINO	

ADDRESS: 1085 SILVER BULLET CT | COUNTY:
CITY: HENDERSON | STATE: NV | ZIP: 890114918 | PHONE: (725)206-8000 | PATIENT LANGUAGE: ENGLISH
AGE: 58Y | D.O.B.: 06/05/1967 | SEX: M | RACE: 5 | MS: S | ETHNICITY: N | PREVIOUS NAME: , | RELIGION: CRT | INFANT AGE:
PT EMPLOYER: Unemployed. | TELEPHONE: | EXT: | OCCUPATION:
EMPLOYER ADDRESS:
CITY: | STATE: | ZIP: | RETIREMENT DATE: // | DISASTER TAG ID:

Contact Information

NEAREST RELATIVE NAME: AJOLO, LULU
RLTN: C | ADDRESS: UNKNOWN
CITY: | STATE: | ZIP: 99999 | PHONE: (702)609-3734 | EXT.:
EMERGENCY CONTACT NAME: BACA, TERESA
RLTN: K | ADDRESS: UNKNOWN
CITY: | STATE: | ZIP: 99999 | PHONE: (725)277-1539 | EXT.:

Guarantor Information

GUARANTOR NAME: AJOLO, ANGEL SABINO | RLTN: S
ADDRESS: 1085 SILVER BULLET CT | PHONE: (725)206-8000
CITY: HENDERSON | STATE: NV | ZIP: 890114918 | OCCUPATION:
GUARANTOR EMPLOYER: Unemployed | PHONE:
ADDRESS:
CITY: | STATE: | ZIP:

Insurance Information

PG: H

INSURANCE NAME 1: HSC/CULINARY | PLAN: I83 | PHONE: (702)733-9938 | POLICY#: 572838762 | GROUP#: | AUTH#: 2615287 | AUTHORIZED BY:
MAILING ADDRESS: PO BOX 211471 | CITY: EAGAN | STATE: MN | ZIP: 55121
SUBSCRIBER NAME: AJOLO, ANGEL SABINO | MAIL TO NAME: HSC/CULINARY, | INS SEX: M | D.O.B.: 19670605 | GNM:
COB: 1 | PAYOR ID: I83 | UR PHONE: | EXT: | INS VERIFIED: V | SUBSCRIBER RLTN: S | NOC REF NUMBER: | NOC REF BY:

INSURANCE NAME 2: | PLAN: | PHONE: | POLICY#: | GROUP#: | AUTH#: | AUTHORIZED BY:
MAILING ADDRESS: | CITY: | STATE: | ZIP:
SUBSCRIBER NAME: | MAIL TO NAME: | INS SEX: | D.O.B.: | GNM:
COB: | PAYOR ID: | UR PHONE: | EXT: | INS VERIFIED: | SUBSCRIBER RLTN: | NOC REF NUMBER: | NOC REF BY:

INSURANCE NAME 3: | PLAN: | PHONE: | POLICY#: | GROUP#: | AUTH#: | AUTHORIZED BY:
MAILING ADDRESS: | CITY: | STATE: | ZIP:
SUBSCRIBER NAME: | MAIL TO NAME: | INS SEX: | D.O.B.: | GNM:
COB: | PAYOR ID: | UR PHONE: | EXT: | INS VERIFIED: | SUBSCRIBER RLTN: | NOC REF NUMBER: | NOC REF BY:

Admitting Information

ADM DATE: 04/14/2026 | TIME: 0944 | SRC: EO | PT TYPE: G | PRI: X | ACCIDENT INFO.: // | VOLUNTARY ADM: | ORGAN DNR: | ISOL: | PT STS: X
ATTENDING PHYSICIAN: Kim Edward | ATTENDING #: 837062 | REF SRC: | ADM BY: VHSCHADD | PREV ADM DATE: | PREVIOUS FACILITY:
ADM DIAGNOSIS: ^cellulitis of right index finger | HOSP SVC: MED | DIAGNOSTIC CODE: | SURGERY DATE: // | BLDLESS: | N/S: ERHD | RM/BED: OBS 03 | Ethnicity: N
ADMITTING PHYSICIAN: Kim Edward | ADMITTING #: 837062 | DISCHARGE DATE: | TIME: | DISCHARGE DISP: | PREVIOUS DISCHARGE DATE:
PROCEDURE: | CLINICAL COMMENT: | MODE OF ARRIVAL / ACCOMPANIED BY: AMB^Ambulance^VHM INVISION
REFERRING PHYSICIAN: | REFERRING #: | EMAIL ADDRESS: AJOLOANGEL1@GMAIL.COM
REL OF INFO: | ADV DIR: | LIVING WILL: | LOCATION OF WILL / DIRECTIVE: | FAMILY DOCTOR: Forte | FAMILY DOCTOR #: 200618 | DIV: VHM

Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106

MEDICAL RECORD

EL0012

UHS-9002
Rev. 04/2025

Patient Identification

1101376356-63844265

AJOLO, ANGEL SABINO
DOB: 06/05/1967 58Y SX: M MED
MRN: 63844265 ADM/REG DT:04/14/2026
Valley Hospital Medical Center

Result type: History & Physical
Result date: April 14, 2026 11:40 PDT
Result status: Auth (Verified)
Result title: Initial Evaluation Note
Performed by: Kim DO, Edward on April 14, 2026 11:40 PDT
Verified by: Kim DO, Edward on April 14, 2026 11:40 PDT
Encounter Info: VHM1101376356, VHM Center, Inpatient, 4/14/2026 -

* Final Report *

Chief Complaint/Reason for Consultation
Pt transferred from Henderson ER - failed outpt tx, right index finger concern for flexor tenosynovitis. Pt given vanco and zosyn at Henderson. Dr. Song requested for transfer to Valley.

History of Present Illness
This is a 58-year-old male with no significant past medical history who presents to Valley Hospital as a transfer from Henderson Hospital for right index finger infection. Patient states symptoms started approximately 1 week ago. Wound reportedly started after he was checking his blood sugar. He then continued more pain and swelling in the area presented into the hospital. On April 11 he was diagnosed with cellulitis and had a I&D in the ED. Patient was ordered Bactrim and Keflex are patient states he never actually started the antibiotics. He continued have worsening pain and swelling and return to Henderson today where he was transferred to Valley Hospital after talking with hand surgery. Patient states he has been feeling generally unwell however denies any significant past medical history. Denies any tobacco, alcohol, and recreational drug use. In the ED hand surgery was consulted.

Review of Systems
14 point ROS negative except stated above

Objective

Measurements (most recent)

Height (cm)	170.18 cm	Chipman-East RN, Crystal	04/14 08:48
Height (inches)	67.00 inch	SYSTEM	04/14 08:48
Weight	72.57 kg	Chipman-East RN, Crystal	04/14 08:48
Weight (lbs)	159.99 lb	SYSTEM	04/14 08:48
Body Mass Index	25.06 kg/m2	Chipman-East RN, Crystal	04/14 08:48

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum
Temp	36.5	L 36.4	L 36.4

Medications
Inpatient Medications
Medications (20) Active
Scheduled: (7)
cefTRIAXone/D5W (Rocephin) 2 gm 50 mL, IV Piggyback, q24H Interval
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
vancomycin/NS (vancomycin) 750 mg 250 mL, IV Piggyback, q12H Interval
Continuous: (1)
Electrolyte (Isolyte-S, Plasma-Lyte) PH7.4 1,000 mL (Isolyte S PH 7.4 1,000 mL) 1,000 mL, IV, 100 mL/hr
PRN: (12)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, q3H
bisacodyl 10 mg Supp (Dulcolax Laxative) 10 mg 1 Supp, Rectal, Daily
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 12.5 gm 25 mL, IV Push, q1H Interval
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
nicotine 7 mg/24 hr Transderm Patch (nicotine 7

	04/14/2026 10:44	04/14/2026 08:58	04/14/2026 08:58
Heart Rate Monitored	L 59 04/14/2026 10:30	L 53 04/14/2026 09:30	66 04/14/2026 09:01
Resp Rate	14 04/14/2026 10:30	14 04/14/2026 10:30	18 04/14/2026 08:48
SBP	120 04/14/2026 10:30	120 04/14/2026 10:30	H 157 04/14/2026 09:01
DBP	78 04/14/2026 10:30	67 04/14/2026 08:48	H 82 04/14/2026 09:01
MAP	92 04/14/2026 10:30	92 04/14/2026 10:30	94 04/14/2026 09:30
SpO2	L 93 04/14/2026 10:30	L 93 04/14/2026 10:30	96 04/14/2026 08:48
O2 Therapy			Room air

Intake and Output (current encounter)

	04/14 08:46 04/14 11:40	04/14 07:00 04/14 11:40	04/13 07:00 04/14 06:59	04/12 07:00 04/13 06:59	04/11 07:00 04/12 06:59
Intake	50	50	0	0	0
Output	0	0	0	0	0
Net Total	50	50	0	0	0

Physical Exam

Assessment/Plan
Diagnoses
Felon of finger (L03.019)
Paronychia of right index finger (L03.011)
1. Cellulitis of right index finger (L03.011)

Assessment:

Physical Exam:
General: In no acute cardiopulmonary distress.
Mental status: Alert. Able to speak full sentences. Oriented x4.
HEENT: Normocephalic. Atraumatic. EOMI. Anicteric.
Respiratory: Normal work of breathing. Normal capillary refill.
Cardiovascular: Regular rhythm. Regular rate. No edema.
Gastrointestinal: Soft, nontender, non-distended. No rigidity or guarding.
Genitourinary: No bladder distension. No suprapubic tenderness.
Neurologic: No focal neurologic deficits. No facial droop. No tremor.
Musculoskeletal: Right index finger with swelling and tenderness to palpation. Range of motion restricted by pain. No obvious fluctuant mass or drainage.

Assessment:

mg/24 hr transdermal film, extended release) 1 Patches, TransDermal, Daily
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	04/14/2026 11:21	04/14/2026 11:21

Results
Recent Labs
General Chemistry

Glucose Level	150 mg/dL (High)	04/14/2026 04:48
Sodium	136 mmol/L (Normal)	04/14/2026 04:48
Potassium	3.5 mmol/L (Normal)	04/14/2026 04:48
Chloride	106 mmol/L (Normal)	04/14/2026 04:48
CO2	24 mmol/L (Normal)	04/14/2026 04:48
Anion Gap	6 mmol/L (Normal)	04/14/2026 04:48
BUN	16 mg/dL (Normal)	04/14/2026 04:48
Creatinine	0.976 mg/dL (Normal)	04/14/2026 04:48
BUN/Creat Ratio	16 (Normal)	04/14/2026 04:48
Calcium	8.7 mg/dL (Normal)	04/14/2026 04:48
Estimated Creatinine Clearance	63.71 mL/min ()	04/14/2026 05:16
eGFR Cr	90 mL/min/1.73m2 (N/A)	04/14/2026 04:48
eGFR Pediatric	Not Reported (N/A)	04/14/2026 04:48
Calc Osmo	276 mOsmol/kg (Normal)	04/14/2026 04:48
Lactic Acid Lvl	0.7 mmol/L (Normal)	04/14/2026 04:48

General Hematology

Cellulitis of right index finger (L03.011)
- *Associated with* ***Felon of finger (L03.019), Paronychia of right index finger (L03.011)***
- Admit to MedSurg

Hand surgery consulted by ED
--MRI hand pending
--IV antibiotics
--Status post I&D bedside on 4/11 at Henderson Hospital
--Further recs pending
Fall precautions, delirium precautions
Supportive care
Close hemodynamic monitoring, telemetry, pulse oximetry, oxygen supplementation if needed
Continue home medications as appropriate
DVT prophylaxis
CBC, renal panel, magnesium, electrolyte protocol
Discussed care plan with patient, labs noted and reviewed
Nursing staff to notify provider regarding abnormal vitals, map<65, fever, change in mentation
PT/OT

Discharge Planning:
ACP:

Discussed face-to-face with patient regarding CODE STATUS/resuscitation status such as in the event of cardiac arrest/respiratory failure, including but not limited to CPR, intubation, pressor support, antiarrhythmics, and all option for life-saving measures were discussed. Risks, benefits and alternatives were explained. Discussion of the patient's relief, values and goals were discussed.

CODE STATUS: Full code

ACP face-to-face time spent on care plan discussion: Approximately 16 minutes

Portions of this record are transcribed using voice recognition software and may contain inadvertent errors with dictation.

Edward Kim DO
Internal Medicine
Platinum Hospitalists
- » **Discharge Planning:**
  - » **Discharge To, Anticipated:** Home independently

WBC	9.22 x10e3/mcL (Normal)	04/14/2026 04:48
RBC	4.64 x10e6/mcL (Normal)	04/14/2026 04:48
Hgb	13.0 gm/dL (Normal)	04/14/2026 04:48
Hct	39.0 % (Normal)	04/14/2026 04:48
MCV	84.1 Femtoliters (Normal)	04/14/2026 04:48
MCH	28.0 pg (Normal)	04/14/2026 04:48
MCHC	33.3 gm/dL (Normal)	04/14/2026 04:48
RDW-SD	40.7 Femtoliters (Normal)	04/14/2026 04:48
RDW-CV	13.2 % (Normal)	04/14/2026 04:48
Plt	333 x10e3/mcL (Normal)	04/14/2026 04:48
MPV	9.3 Femtoliters (Normal)	04/14/2026 04:48
Neut % Auto	47.6 % (N/A)	04/14/2026 04:48
Lymph % Auto	25.9 % (N/A)	04/14/2026 04:48
Mono % Auto	7.3 % (N/A)	04/14/2026 04:48
Eos % Auto	17.1 % (N/A)	04/14/2026 04:48
Baso % Auto	1.7 % (N/A)	04/14/2026 04:48
Immature Grans %	0.4 % (Normal)	04/14/2026 04:48
Neut # Auto	4.38 x10e3/mcL (Normal)	04/14/2026 04:48
Lymph # Auto	2.39 x10e3/mcL (Normal)	04/14/2026 04:48
Mono # Auto	0.67 x10e3/mcL (Normal)	04/14/2026 04:48
Eos # Auto	1.58 x10e3/mcL (High)	04/14/2026 04:48
Baso # Auto	0.16 x10e3/mcL (High)	04/14/2026 04:48
Immature Grans # Auto	0.04 x10e3/mcL (Normal)	04/14/2026 04:48

Other Chemistry

CRP	0.94 mg/dL (High)	04/14/2026 04:48

Abnormal Labs
General Chemistry

Glucose Level	150 mg/dL (High)	04/14/2026 04:48

General Hematology

Eos # Auto	1.58 x10e3/mcL (High)	04/14/2026 04:48
Baso # Auto	0.16 x10e3/mcL (High)	04/14/2026 04:48

Other Chemistry

CRP	0.94 mg/dL (High)	04/14/2026 04:48

Prenatal Labs
No Data Available

Transcribed Prenatal Labs
No Data Available

Signature Line
Electronically Signed By: Kim, Edward DO
On: 04.14.2026 11:40 PDT

Completed Action List:
* Perform by Kim DO, Edward on April 14, 2026 11:40 PDT
* Sign by Kim DO, Edward on April 14, 2026 11:40 PDT
* VERIFY by Kim DO, Edward on April 14, 2026 11:40 PDT

Result type:	Discharge Summary
Result date:	April 17, 2026 13:06 PDT
Result status:	Modified
Result title:	Discharge Note
Performed by:	Kim DO, Edward on April 17, 2026 13:06 PDT
Verified by:	Kim DO, Edward on April 17, 2026 13:06 PDT
Encounter info:	VHM1101376356, VHM Center, Inpatient, 4/14/2026 -

* Final Report *

Document Has Been Revised

Patient Information
Attending Physician: Kim DO, Edward
Admitting Diagnosis: Cellulitis of right index finger
Discharge Location: VHM ERHD
Primary Care Physician: Forte, Family Practice MD
Inpatient Admit Date/Time: 04/14/2026 09:44

Discharge Diagnosis
Cellulitis of right index finger (L03.011)
Felon of finger (L03.019)
Finger pain-swelling (8BCBAA61-E71A-4C1A-897C-674EDD5380DE)
Paronychia of right index finger (L03.011)
Cellulitis of right index finger (L03.011)
Abscess of index finger (L02.519)
Prediabetes (R73.03)

–

Discharge Medications
acetaminophen (acetaminophen 325 mg oral tablet) 650 Milligram 2 Tabs By Mouth Every 4 hours as needed Pain 1 - 3 (Mild) prn painnot to exceed 4000 mg/day
amoxicillin-clavulanate (Augmentin 875 mg-125 mg oral tablet) 1 Tabs By Mouth Every 12 hours for 14 Days
doxycycline (doxycycline hyclate 100 mg oral tablet) 100 Milligram 1 Tabs By Mouth Every 12 hours for 14 Days

Inpatient Medications
Medications (15) Active
Scheduled: (2)
cefTRIAXone/D5W (Rocephin) 2 gm 50 mL, IV Piggyback, q24H Interval
vancomycin/NS (vancomycin) 750 mg 250 mL, IV Piggyback, q12H Interval
Continuous: (0)
PRN: (13)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, q3H
bisacodyl 10 mg Supp (Dulcolax Laxative) 10 mg 1 Supp, Rectal, Daily
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 12.5 gm 25 mL, IV Push, q1H Interval
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
HYDROcodone -acetaminophen 5 mg-325 mg tab (Norco 5 mg-325 mg oral tablet) 1 Tabs, Oral, q6H While Awake

labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
nicotine 7 mg/24 hr Transderm Patch (nicotine 7 mg/24 hr transdermal film, extended release) 1 Patches, TransDermal, Daily
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

Follow-Up/Discharge Instructions
Follow-up
Wound Care (Daily): 1. Cleanse with wound cleaner 2. Apply xeroform, 3. Cover with 4 x 4 gauze 5. Wrap with coban,
Song MD, Walter, ORT, SUR, In 4 days 04/21/26
Return to Emergency Department, only if needed - 03-Return immediately if symptoms worsen
Forte, Family Practice MD, MED, Within 1-2 days
Follow up with primary care provider, Within 2-4 weeks

Education
Abscess, Incision And Drainage
CELLULITIS
Wound Care

Post Discharge Care
Discharge Request
- 04/17/26 10:06:00 PDT, Home Routine
- **Order Comment:**

Pending Results
CBC with Diff ordered on 04/15/2026
Comprehensive Metabolic Panel ordered on 04/15/2026
Magnesium Level ordered on 04/15/2026
Vancomycin Trough ordered on 04/17/2026

Results
Discharge Labs
General Chemistry

Glucose Level	160 mg/dL (High)	04/17/2026 08:58	**Sodium**	136 mmol/L (Normal)	04/17/2026 08:58
Potassium	4.5 mmol/L (Normal)	04/17/2026 08:58	**Chloride**	104 mmol/L (Normal)	04/17/2026 08:58
CO2	25 mmol/L (Normal)	04/17/2026 08:58	**Anion Gap**	7 mmol/L (Normal)	04/17/2026 08:58
BUN	10 mg/dL (Normal)	04/17/2026 08:58	**Creatinine**	0.88 mg/dL (Normal)	04/17/2026 08:58
BUN/Creat Ratio	11 (Normal)	04/17/2026 08:58	**Calcium**	9.6 mg/dL (Normal)	04/17/2026 08:58
Albumin. Level	4.1 gm/dL (Normal)	04/17/2026 08:58	**TP**	7.3 gm/dL (Normal)	04/17/2026 08:58
A/G Ratio	1.3 (Normal)	04/17/2026 08:58	**T Bill**	0.3 mg/dL (Normal)	04/17/2026 08:58
Alk Phos	57 units/L (Normal)	04/17/2026 08:58	**AST**	22 units/L (Normal)	04/17/2026 08:58
ALT	19 units/L (Normal)	04/17/2026 08:58	**Mg Lvl**	2.2 mg/dL (Normal)	04/17/2026 08:58
Phosphorus	2.2 mg/dL (Low)	04/14/2026 12:28	**Estimated Creatinine Clearance**	85.55 mL/min ()	04/17/2026 10:13
eGFR Cr	>90 mL/min/1.73m2 (N/A)	04/17/2026 08:58	**eGFR Pediatric**	Not Reported (N/A)	04/17/2026 08:58
Hgb A1C	6.3 % (High)	04/14/2026 20:13			

General Hematology

WBC	9.42 x10e3/mcL (Normal)	04/17/2026 08:58	**RBC**	4.86 x10e6/mcL (Normal)	04/17/2026 08:58

Hgb	13.6 gm/dL (Normal)	04/17/2026 08:58	**Hct**	41.6 % (Normal)	04/17/2026 08:58
MCV	85.6 Femtoliters (Normal)	04/17/2026 08:58	**MCH**	28.0 pg (Normal)	04/17/2026 08:58
MCHC	32.7 gm/dL (Normal)	04/17/2026 08:58	**RDW-SD**	41.9 Femtoliters (Normal)	04/17/2026 08:58
RDW-CV	13.4 % (Normal)	04/17/2026 08:58	**Plt**	347 x10e3/mcL (Normal)	04/17/2026 08:58
MPV	9.4 Femtoliters (Normal)	04/17/2026 08:58	**Neut % Auto**	59.1 % (N/A)	04/17/2026 08:58
Lymph % Auto	19.6 % (N/A)	04/17/2026 08:58	**Mono % Auto**	4.0 % (N/A)	04/17/2026 08:58
Eos % Auto	14.9 % (N/A)	04/17/2026 08:58	**Baso % Auto**	1.8 % (N/A)	04/17/2026 08:58
Immature Grans %	0.6 % (Normal)	04/17/2026 08:58	**Neut # Auto**	5.56 x10e3/mcL (Normal)	04/17/2026 08:58
Lymph # Auto	1.85 x10e3/mcL (Normal)	04/17/2026 08:58	**Mono # Auto**	0.38 x10e3/mcL (Normal)	04/17/2026 08:58
Eos # Auto	1.40 x10e3/mcL (High)	04/17/2026 08:58	**Baso # Auto**	0.17 x10e3/mcL (High)	04/17/2026 08:58
Immature Grans # Auto	0.06 x10e3/mcL (Normal)	04/17/2026 08:58			

HIV

HIV 1/O/2	Non-Reactive (Normal)	04/15/2026 20:05			

Hepatitis

Hep A IgM	Non-Reactive (Normal)	04/15/2026 20:05	**Hep B Core IgM**	Non-Reactive (Normal)	04/15/2026 20:05
Hepatitis B Surface Antigen	Non-Reactive (Normal)	04/15/2026 20:05	**Hep C Ab**	Non-Reactive (Normal)	04/15/2026 20:05

Other Chemistry

Procalcitonin Lvl	0.07 ng/mL (High)	04/14/2026 20:13	**CRP**	<0.50 mg/dL (Normal)	04/17/2026 08:58

Other Hematology

Sed Rate	25 mm/hr (High)	04/14/2026 20:13			

Other Microbiology

MRSA by PCR	Not Detected (Normal)	04/15/2026 21:19			

Sensitive Infectious Diseases

Syph IgG with rflx	Non-Reactive (Normal)	04/15/2026 20:05			

Therapeutic Drug Monitoring

Vancomycin Trough	19.4 mcg/mL (Normal)	04/17/2026 08:58	**Vancomycin Peak**	14.6 mcg/mL (Low)	04/15/2026 09:47

Thyroid

TSH Ultrasensitive	0.431 mc Intl units/mL (Low)	04/14/2026 12:28			

Objective

Measurements (most recent)

Height (cm)	170.18 cm	Chipman-East RN, Crystal	04/14 08:48
Height (inches)	67.00 inch	SYSTEM	04/14 08:48
Weight	72.57 kg	Chipman-East RN, Crystal	04/14 08:48
Weight (lbs)	159.99 lb	SYSTEM	04/14 08:48
Body Mass Index	25.06 kg/m2	Chipman-East RN, Crystal	04/14 08:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.8	04/17/2026 11:12	**L** 36.3	04/16/2026 15:04	**L** 36.3	04/16/2026 15:04
Peripheral Pulse Rate	69	04/17/2026 11:12	**L** 54	04/16/2026 15:04	69	04/17/2026 11:12
Resp Rate	18	04/17/2026 11:12	16	04/16/2026 23:45	18	04/16/2026 15:04
SBP	129	04/17/2026 11:12	111	04/17/2026 06:12	129	04/16/2026 15:04
DBP	76	04/17/2026 11:12	67	04/16/2026 15:04	**C** 92	04/17/2026 04:40
MAP	89	04/17/2026 11:12	80	04/16/2026 15:04	98	04/17/2026 04:40
SpO2	95	04/17/2026 11:12	94	04/16/2026 15:04	98	04/17/2026 04:40
O2 Therapy	Room air		Room air		Room air	

Intake and Output (current encounter)

	04/14 08:46 04/17 13:06	04/17 07:00 04/17 13:06	04/16 07:00 04/17 06:59	04/15 07:00 04/16 06:59	04/14 07:00 04/15 06:59
Intake	630	0	50	530	50
Output	0	0	0	0	0
Net Total	630	0	50	530	50
Urine Count	4	0	0	3	1

Hospital Course

This is a 58-year-old male with no significant past medical history who presents to Valley Hospital as a transfer from Henderson Hospital for right index finger infection. Patient states symptoms started approximately 1 week ago. Wound reportedly started after he was checking his blood sugar. He then continued more pain and swelling in the area presented into the hospital. On April 11 he was diagnosed with cellulitis and had a I&D in the ED. Patient was ordered Bactrim and Keflex are patient states he never actually started the antibiotics. He continued have worsening pain and swelling and return to Henderson today where he was transferred to Valley Hospital after talking with hand surgery. Patient states he has been feeling generally unwell however denies any significant past medical history. Denies any tobacco, alcohol, and recreational drug use. In the ED hand surgery was consulted.

Patient underwent a bedside I&D with orthopedic surgery on 4/14. Purulent discharge was noted coming from the eponychial fold. ID was consulted. Blood cultures remain negative. Wound culture showed mixed growth gram-positive flora. Was cleared for discharge by orthopedic surgery recommended outpatient follow-up. Cleared for discharge by ID who recommended Augmentin and doxycycline for 2 weeks. Medications were sent to his pharmacy.

Information provided in discharge paperwork. All appropriate refills on medications as well as new prescriptions provided to the patient. Patient educated on importance of medication adherence and reason for medication administration. Given strict return precautions. The patient has no other complaints at this time. All questions addressed at the bedside.

On discharge, patient has stable vitals, stable labs, and acceptable physical exam. Patient understands that should symptoms worsen, it is advisable to follow up with a PCP and to go to the ED for medical emergencies. Patient is safe for discharge at this time. Patient requesting discharge and to forgo further evaluation.

More than 30 minutes was spent with the patient on counseling, review of medication, evaluation, and examination during this discharge.

Edward Kim, DO
Platinum Hospitalists

Disclaimer: This note was dictated by speech recognition. Effort has been made to ensure accuracy of this report, however occasional wording/dictation errors may be present.

Signature Line
Electronically Signed By: Kim, Edward DO
On: 04.17.2026 13:06 PDT

Electronically Signed On: 04.17.2026 13:08 PDT
Kim, Edward DO

Completed Action List:
* Perform by Kim DO, Edward on April 17, 2026 13:06 PDT
* Sign by Kim DO, Edward on April 17, 2026 13:06 PDT
* VERIFY by Kim DO, Edward on April 17, 2026 13:06 PDT
* Modify by Kim DO, Edward on April 17, 2026 13:08 PDT
* Sign by Kim DO, Edward on April 17, 2026 13:08 PDT

Result type:	Procedure Note
Result date:	April 14, 2026 18:35 PDT
Result status:	Unauth
Result title:	Bedside I&D
Performed by:	Parry DO, Connor on April 14, 2026 18:37 PDT
Encounter info:	VHM1101376356, VHM Center, Inpatient, 4/14/2026 -

* Preliminary Report *

Procedure Name

Right Index finger distal phalanx bedside I&D

Consent
Operative and nonoperative indications were discussed with the patient. The patient is amenable to bedside I&D. The risk, benefits, and complications of the procedure be performed were discussed at length with the patient. Risk include but not limited to pain, bleeding, infection, damage neurovascular structures, damage to tendinous or ligamentous tissues, need for reoperation, continued swelling at the hand, chronic pain, scarring, contracture, stiffness, amongst many others. There is of local anesthesia were additionally discussed to include pain, persistent numbness. With all these things in mind the patient elected to proceed with right index finger distal phalanx bedside I&D

Indication
Right Index finger infection

Sedation
5 cc 1% plain lidocaine

Complications
None

Technique
Patient was greeted at the bedside and appropriate consents were obtained as above. The right hand and index finger were cleaned with a chlorhexidine solution. The MCP volar crease was injected with 5 cc of 1% plain lidocaine. After adequate anesthesia the area was then cleaned again with additional chlorhexidine solution. Proper sterile technique was used. First using a Freer, the eponychial fold was evacuated. Purulent discharge noted to be coming from eponychial fold. The fingernail was removed of the index finger for proper evaluation. Sharp dissection was used using a 15 blade to debride any fibrinous material in the center of the wound bed. Dead skin was trimmed around the wound edges with sharp scissors. Afterwards sterile dressing using Xeroform, 4 x 4's, Kerlix, and Ace wrap was placed. Patient tolerated this well.

Estimated Blood Loss
1 cc

Completed Action List:

* Perform by Parry DO, Connor on April 14, 2026 18:37 PDT
* Sign by Parry DO, Connor on April 14, 2026 18:37 PDT

P AUDLO, ANGEL SABINO - Order Information For: Consult to Case Management (Case Management Consult)

Task View Options Help

Original order entered and electronically signed by Thomas RN, Caitlin on 4/17/2026 at 13:27 PDT.
Phone w/ Readback (Cosign) order by Kim DO, Edward
Consults Department

Consult to Case Management (Case Management Consult)

Details Additional Info History Comments Validation Results Ingredients Pharmacy

- Modify 4/17/2026 13:33 PDT
- Order 4/17/2026 13:27 PDT

Modify 4/17/2026 13:33 PDT

Entered and electronically signed by Thomas RN, Caitlin.
Phone w/ Readback (Cosign) order by Kim DO, Edward

Details	After	Before
Isolation		No Isolation

Comment

After
Wound Care to Right Index Finger (Daily): 1. Cleanse with wound cleaner 2. Apply xeroform, 3. Cover with 4 x 4 gauze 5. Wrap with coban

Before

Validations

Cosignature
Routed to Kim DO, Edward

Order 4/17/2026 13:27 PDT

Entered and electronically signed by Thomas RN, Caitlin.
Phone w/ Readback (Cosign) order by Kim DO, Edward

Status

Order Status	Ordered
Department Status	Ordered

Details

Requested Start Date/Time	4/17/2026 13:27 PDT
Priority	Routine
Reason for Consult+	D/C to Home w Home Health
Isolation	No Isolation
Special instructions	HHC needed for daily wound care dressings

Validations

Cosignature
Routed to Kim DO, Edward